

The California Baseline

What every California Medi-Cal paper inherits: the estate as it actually runs, the obligations already on the calendar, the money as it actually works, and the limits none of our papers may exceed.

This is a reference document, not an argument. It exists because four California papers were written in parallel and each described the same operating environment differently — or, more often, not at all. That is how a firm ends up contradicting itself in front of a client.

Everything here is shared. A paper that needs the enrollment split, the fiscal intermediary's name, the January 2027 convergence, the federal match mechanics, or the due-process floor takes it from this document by section number and does not restate it. What a paper adds is its own thesis, its own use case, and its own proof.

The discipline that matters most: **where a fact is not publicly sourceable, this document says so rather than estimating.**

Several contract terms below are marked *DHCS to supply*. Those marks are not gaps to be filled with plausible numbers — they are the first data request of any engagement.

READING MARKS

Confirmed — a primary source was retrieved and read; the link is given.

Unverified — asserted by a secondary source, an aggregator, or inference. Usable with the mark attached; never as a bare fact.

DHCS to supply — not published anywhere. Do not estimate it.

THE ELEVEN CHAPTERS

1 · How this POV is used

The contract between this document and the papers. Naming authority.

2 · Medi-Cal at a glance

The denominators every paper must use, and the split everyone misreads.

3 · The estate map

System, operator, contract, owner. The load-bearing exhibit.

4 · How work flows today

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5 · The mandate clock

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What is exchanged today and what is owed by January 2027.

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One model. The others become views of it.

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What no California paper may claim, and the deferred candidates.

§1 · HOW THIS POV IS USED

The contract between this document and the papers

THREE RULES

Inherit, don't restate. A paper cites this document by section. It does not reproduce the estate map, re-derive the enrollment split, or re-argue the legal floor. Where a paper needs a fact to be visible on the page, it prints the fact and the POV section, not the derivation.

One name per thing. Four papers currently use four names for the same asset. This document names it once and the other names are retired. Where a paper's own vocabulary is its contribution — a ladder, a tier, a gate — that vocabulary stays with the paper and is not promoted here.

Contradiction is a defect, not a difference of view. If a paper needs to depart from this baseline, it says so explicitly and gives its reason in the text. Silent divergence between two documents going to the same client is the failure this POV exists to prevent.

NAMING AUTHORITY

USE THIS	RETIRED
Medi-Cal evidence record	Evidence fabric · neutral evidence record · RECORD-A
Fiscal intermediary (FI)	"the vendor" · "the contractor" unqualified
CMS's successor MITA framework	"MITA 3.0" as a name for the future state — MITA 3.0 is the <i>current published</i> framework

THE MITA NAMING CORRECTION, IN FULL

MITA 3.0 is the framework CMS published and states assess against today. The successor is MITA 4.0, and CMS has been rebuilding it since 2023 around outcomes and metrics with capabilities defined by outcomes, roles, business processes, information and technology. **The maturity-model page in CMS's own MITA 4.0 repository still reads *Under Construction*.**

So no California paper should coin "MITA 3.0" for a future state. The defensible framing is that advancing in MITA maturity remains a condition of the 90 percent match under 42 CFR § 433.112(b)(11), that CMS's live operational guidance has moved to outcomes and metrics, and that California can arrive at the successor with its instrument already running.

WHAT STAYS WITH THE INDIVIDUAL PAPERS

Its thesis · its one or two use cases · its own vocabulary and any instrument it proposes · its proof, worked end to end · its ask. Everything else should be here.

§2 · MEDI-CAL AT A GLANCE

The denominators, and the split everyone misreads

FIGURE	VALUE	SOURCE
Certified eligibles, April 2026	13,910,180	DHCS Fast Facts, Jul 2026 · <i>confirmed</i>
In managed care	13,207,633 (94.9%)	same
Fee-for-service	702,547 (5.1%)	same
Weekly checkwrite, FY2017-18 average	\$345m (~\$17.9bn/yr)	DHCS fact sheet · <i>confirmed</i>
FFS PA requests, CY2025 — medical & behavioral	78,987	DHCS PA metrics · <i>confirmed</i>
FFS PA requests, CY2025 — CCS	755,850	DHCS PA metrics · <i>confirmed</i>
FFS PA requests, CY2025 — dental	3,138,788	DHCS PA metrics · <i>confirmed</i>
FFS claim volume · suspense queue volume	DHCS to supply	not published

THE 95/5 TRAP

The managed-care share is an **enrollment** split. It is not a **transaction** split, and treating it as one is the most common error in Medi-Cal strategy work.

The FFS pipeline still carries all outpatient pharmacy billed on a pharmacy claim, FFS dental, Medicare crossovers, carve-out benefits, retroactive and pre-assignment coverage, and county specialty behavioral health claiming.

Two parallel economies: **a fee-for-service economy DHCS owns and sees completely**, and an **encounter economy covering 95% of members that DHCS does not adjudicate.**

§3 · THE ESTATE MAP

What runs Medi-Cal, who operates it, under what contract

A capability model that cannot be placed on this table is a taxonomy; a recommendation that cannot name the contract it changes is a wish.

SYSTEM	WHAT IT DOES	OPERATOR	CONTRACT	ACCOUNTABLE OWNER	STATUS
CA-MMIS	FFS claims adjudication, provider payment, eligibility inquiry, provider portal, service centre	Gainwell Technologies as fiscal intermediary since 1 Oct 2020 (confirmed)	Two contracts — FI Business Operations (Gainwell) and FI Maintenance & Operations (incumbent named as IBM in an aggregator parse of the gated RFI). <i>Terms DHCS to supply</i>	DHCS CA-MMIS Operations	In service. FI-M&O re-procurement opened April 2024 and has published nothing since
eTAR	Prior authorization for FFS services. All inpatient stays require a TAR	Rides the FI-operated provider portal, inside the CA-MMIS estate. Adjudicated by DHCS staff	No separate contract; a CA-MMIS component	DHCS Clinical Assurance Division — 5 field offices plus 2 pharmacy sections	In service. FFS inpatient UM runs under a §1903(i)(4) Superior Systems Waiver to 30 Sep 2029
PAVE	Provider enrollment, revalidation, change reporting. ~313,900 active providers	Vendor-operated SaaS. <i>DHCS names no vendor publicly</i>	Existing: <i>not sourceable</i> . Replacement RFI 24-027 contemplates 5 years + two 1-year options	DHCS Provider Enrollment Division	In service, being replaced. Successor at CDT Project Approval Lifecycle Stage 1
MEDS	Statewide eligibility data hub; state system of record for eligibility. Live since 1983 ; ~35,000 users	DHCS-operated . No vendor operator identified	State-run; none published	DHCS Enterprise Technology Services; policy via Medi-Cal Eligibility Division	Modernization stalled . Began 2016; absent from the current CDT project list
CalSAWS	County eligibility and case management. System of record for all Medi-Cal cases . Public portal is BenefitsCal	Gainwell (infrastructure) and Deloitte (maintenance) since Feb 2024, succeeding Accenture	Contracted by the CalSAWS JPA, not DHCS . Up to \$1.33bn/11yrs (unverified)	CalSAWS Consortium — a JPA of all 58 counties. DHCS is a program partner	Consolidation complete. All 58 counties live 30 Oct 2023
CalHEERS	Consumer portal and business rules engine. Determines MAGI Medi-Cal , APTC, CSR and QHP eligibility	Deloitte under the SDMO contract since 2020, succeeding Accenture	SDMO expires June 2027 . Successor procured by CalHHS OTSI, not DHCS	Two sponsors — Covered California and DHCS. Project office CalHHS OTSI	In service; re-procurement live now
Medi-Cal Rx	Statewide FFS pharmacy benefit — claims and PA for drugs billed on a pharmacy claim	Prime Therapeutics State Government Solutions, successor to Magellan; cutover 25 Mar 2024	RFP 19-96125, awarded Dec 2019. <i>Term and expiry DHCS to supply</i>	DHCS Pharmacy Benefits Division	In service since 1 Jan 2022
PACES	Receives managed care encounter data — X12 837 and NCPDP	DHCS-operated . No vendor named publicly	None published	DHCS Health Information Management Division — Data Quality Reporting Unit	In service since 2015, replacing PCES
CD-MMIS Medi-Cal Dental	Dental claims, dental TARs, customer service for Dental, CCS, GHPP	Gainwell as Dental Business Operations intermediary, replacing Delta Dental	RFP 20-10354, intent to award Jul 2022. <i>Term DHCS to supply</i>	DHCS Medi-Cal Dental Services Division	Recently replaced — transition executed 13 May 2024 , ~7 months late
Behavioral health estate	Multiple separate collection and reporting systems — BHIS, CalOMS Tx, DATAR among them	Not published	Consolidation would integrate 12 systems (LAO said "at least 11"). No vendor, funding or schedule published	DHCS Behavioral Health / EDIM	Status contested . Absent from the current CDT project list

DHCS OPERATES DIRECTLY

MEDS · PACES · behavioral health reporting, eTAR and

The existing eTAR adjudication.

Changeable by internal decision plus CDT approval — but all are

A VENDOR OPERATES UNDER DHCS CONTRACT

CA-MMIS incl. eTAR and the provider portal · PAVE · Medi-Cal Rx · CD-MMIS. Reachable, but

OUTSIDE DHCS CONTROL

CalSAWS and BenefitsCal (58-county JPA) · **CalHEERS** (OTSI, Covered California co-sponsor).

Any use case touching county

§3.2

The fiscal intermediary lineage

Worth getting exactly right, because a DHCS reader knows it and a wrong version discredits everything downstream.

WHEN	WHAT HAPPENED
~1987–2008	EDS operates CA-MMIS; the system was built in the late 1970s
2008	HP acquires EDS
18 Mar 2010	ACS State Healthcare executes the FI agreement, having won the 2007 bid
2010	Xerox, having acquired ACS, holds the contract — \$1.7bn, expiry 30 Jun 2016
30 Sep 2011	Legacy takeover completed
Dec 2014	Health Enterprise Release 1.0
11 Apr 2016	Settlement — development abandoned ; Xerox to operate and maintain through 30 Sep 2019
Jan 2017	Xerox's BPO business spins out as Conduent
1 Oct 2019	DXC Technology assumes all FI duties — the hop most accounts omit
1 Oct 2020	DXC sells its US state and local health business to Veritas Capital; the FI becomes Gainwell

The last hop was a **corporate carve-out, not a procurement**. Anyone writing "DHCS re-procured to Gainwell in 2020" is wrong, and the 2016 settlement — a modernization abandoned after six years — is the reason a DHCS audience is sceptical of platform arguments. Any California paper that proposes a build should acknowledge it once, early, without being asked.

§3.3

Contract and procurement windows

The dates on which change is actually possible. A recommendation that misses its window becomes a change order, which is where efficiency accrues to the vendor rather than the state.

WINDOW	WHAT
Open now	CA-MMIS FI-M&O . RFI 23-072 released 29 Apr 2024 and laid out an RFP in Q3 2024, execution Q3 2025, takeover ending Q3 2026. None of it happened publicly; no RFP has been found as of 27 Aug 2026.
June 2027	CalHEERS SDMO expires. Successor RFP posted Sep 2025 through OTSI — five base years plus five options, so the successor could run to 2037
30 Jun 2028	CA-MMIS FI Business Operations (Gainwell) stated expiry — single aggregator source, <i>unverified</i>
~2026–28	PAVE successor (Medi-Cal PEOA) — 5 years plus two options contemplated; no RFP or award found
30 Sep 2029	Superior Systems Waiver — the federal authority under which California runs FFS inpatient utilization management
~2030–35	CalSAWS infrastructure and maintenance contracts — <i>execution dates never published</i>
Unknown	Medi-Cal Rx and CD-MMIS Dental — both awarded, neither term published

THE FINDING TO LEAD WITH

California is between contracts on the system that determines whether the state can ever see its own claims decisions, and **the re-procurement has been open and silent for over two years**. An evidence or provenance requirement costs almost nothing inside a solicitation and almost everything as a change order afterwards. That asymmetry is the most actionable fact in this document.

NOT PUBLICLY SOURCEABLE — THE STANDING DATA REQUEST

Every current FI contract term · the FI-BO / FI-M&O split as a stated fact · PAVE's vendor · Medi-Cal Rx term and expiry · CD-MMIS term and value · whether PACES is vendor-built · the behavioral health consolidation's scope, funding and schedule · MEDS Modernization's current state · CalSAWS execution dates. **Ask for these; do not model around estimates of them.**

§4 · HOW WORK FLOWS TODAY

The fee-for-service claim, end to end

Described with its real artifacts, because those artifacts are what any proposal must actually change.

THE PIPELINE

Five stages, and in business-architecture terms a document-imaging factory bolted to a mainframe rules engine.

1 · Pre-sort and acceptance. USPS pre-sorts by PO box; the FI mailroom examines each claim for acceptability. Paper goes to West Sacramento; **walk-up delivery is not accepted.**

2 · Scan and control number. Each claim receives a unique **13-digit Claim Control Number containing the Julian date of receipt.** The CCN is the identity spine of everything downstream — what a Claims Inquiry Form traces, what a remittance line reports, what an appeal references.

3 · Data entry. Neatly typed forms route to OCR; the rest go to **Key Data Entry** — a human retyping a claim.

4 · Edits and audits. Line-by-line verification against eligibility, procedure compatibility, duplication and authorization. Inpatient claims are processed whole rather than line-by-line.

5 · Payment or suspension. Claims passing every check route to the State Controller's Office. Failed claims suspend or deny.

THE ARCHITECTURAL FACT

The unit of work is a **form image with a control number**, not a structured transaction with a state machine. Every proposal about automation has to survive that sentence: the canonical artifact is a scanned image and the exception path is a human reading it.

SUSPENSE — WHERE THE HUMANS ARE

Claims failing edits suspend for examiner review. Examiners identify errors, review scanned images and attachments, and correct input errors; cases requiring medical judgment go to a physician or other qualified professional under Title 22.

Two properties matter. **Suspense is visible but not actionable to the provider** — suspended claims appear on the remittance advice indefinitely, and the manual instructs providers *not* to submit an inquiry for them. The provider is told to wait, with no committed resolution date. And **suspense holds two different human roles**: clerical correction of data-entry and edit failures, and licensed clinical judgment. They are different automation problems with different evidence requirements and should never be treated as one queue.

THE REMEDIES, AND THEIR CLOCKS

INSTRUMENT	CLOCK
Original claim	6 months from end of month of service. Months 7–9 pay at 75% ; months 10–12 at 50% ; after 12, denied (W&I §14115)
Claims Inquiry Form	6 months from the remittance date. Requires the signed original, mailed — no electronic channel is described
Appeal (form 90-1)	90 days. Acknowledged in 15 days, decided in 45, plus 30 more if it goes to professional review (22 CCR §51015)
Federal standard	90% of <i>clean</i> claims in 30 days, 99% in 90 (42 CFR §447.45)

Note the asymmetry. The federal standard is a percentile on the clean-claim population, and a suspended claim is by construction not clean.

The pend queue absorbs the timeliness risk of the entire system — which is why its size and cycle time have never had to be managed as first-class quantities.

CHECKWRITE — THE EXTERNAL DEPENDENCY

Weekly, not monthly. Files reach the **State Controller's Office** on Mondays; the SCO mails warrants three days later; EFT settles three to five days after that. The money leaves through a separate constitutional office, on the SCO's calendar, and **the SCO — not the FI — generates the warrant and the remittance advice.** Any latency claim downstream of that handoff is not DHCS's to make.

THE REMITTANCE ADVICE

Produced by the SCO from FI payment data, reporting in fixed order — adjustments, approves, denies, suspends. **A denial message on the remittance is the only record of a claim denial**; there is no separate notice. And the remittance date is the clock-start for every downstream remedy, which makes it the single most load-bearing timestamp in the estate.

SHARE OF COST

A per-member monthly deductible set by county welfare departments, cleared by the **provider** in a real-time transaction at the point of service, with irreversible commitment semantics once certified. A claim for a member whose share of cost is not certified **is denied**. It is the clearest illustration that Medi-Cal claims processing is not one process but a mesh of coupled processes with different owners.

§4.2

Authorization — the TAR estate

Prior authorization sits upstream of adjudication and gates it. All inpatient stays require a Treatment Authorization Request. Providers submit through **eTAR** — mandatory by regulation since 2017 — or on paper to the processing centre in West Sacramento. **Adjudication is by DHCS staff**, not the fiscal intermediary: designated Medi-Cal consultants across five field offices plus two pharmacy sections.

WHAT CY2025 ACTUALLY LOOKED LIKE

MEDI-CAL, MEDICAL AND BEHAVIORAL**CY2025**

Total requests	78,987
Approved / denied / modified	71% / 28% / 1%
Average / median decision	4 / 2 calendar days
Decided within 7 days	62%
Auto-denied after extension	9,439 (12%)
Appeals filed / approved	43 / 3

Read the 12% carefully. Roughly one in eight requests is not decided on the merits — it times out into denial when requested documentation does not arrive. Those 9,439 are **42% of all denials**, and they are an information-logistics failure rather than a clinical judgment. It is the largest addressable defect visible in published data.

Two cautions. The published appeal line states 3 of 43 as 14%, which does not reconcile to 7% — do not quote it as a rate. And volume sits elsewhere: **dental carried 3,138,788 requests and California Children's Services 755,850**, against 78,987 for medical and behavioral. Any claim that fee-for-service authorization is low-volume is wrong.

Nothing below the total is published — not by service category, procedure code, provider type, field office, or channel. Deferral counts distinct from auto-denials do not exist publicly. That is the first data pull of any authorization engagement.

§4.3

Eligibility — who decides what

The division of labour, precisely, because papers get this wrong and it decides which use cases are even reachable:

- A county worker enters the application into **CalSAWS**.
- CalSAWS calls the **CalHEERS business rules engine** over the eHIT interface.
- **CalHEERS determines MAGI Medi-Cal**, along with premium tax credits and marketplace eligibility.
- **CalSAWS remains the system of record for all Medi-Cal cases**, and non-MAGI determinations are made there by county staff.
- **MEDS** is the state system of record downstream of both.

Consequence. Determination authority is distributed across 58 county agencies operating one shared system under a joint powers authority. This is a *governance* fragmentation, not a systems fragmentation — the consolidation finished in October 2023. **DHCS does not control the eligibility determination path**, and any paper proposing to automate it is proposing something DHCS cannot start alone.

§4.4

Encounters — the derived artifact

A managed care plan pays on its own record and *then* reports. The encounter is therefore **a derived artifact, not the transaction itself**. Plans submit X12 837 and NCPDP files to **PACES** over secure file transfer; DHCS accepts or rejects; rejections loop back on a batch cadence.

Three consequences follow, and every managed-care argument in every paper rests on them. The **latency of the correction loop is the latency of the state's own view of what care was delivered**. The disagreement between what a plan adjudicated and what DHCS accepted is **visible only in aggregate**, never adjudicated record by record under a versioned rule. And incomplete or late encounters **propagate forward into capitation rates**, which is why this is the most expensive data-quality problem in the program.

§5 · THE MANDATE CLOCK

What is already on the calendar

Single-owner. Where papers have disagreed on a date, this table is the one that governs.

FEDERAL

DATE	OBLIGATION
1 Jan 2026 in force	CMS-0057-F prior-auth decision timeframes — 72 hours expedited, 7 calendar days standard — and a specific reason on every denial
16 Feb 2026 in force	42 CFR Part 2 compliance — single consent, HIPAA-style breach notification on substance use disorder records
31 Mar 2026 met	First annual public prior-authorization metrics. DHCS published three reports in April 2026
1 Jan 2027	Four CMS-0057-F APIs live — Patient Access with PA data, Provider Access, Payer-to-Payer, Prior Authorization — for DHCS fee-for-service and for every plan
1 Jan 2027	P.L. 119-21 §§71103, 71104, 71107, 71112, 71119 — community engagement verification, six-month redeterminations for the adult group, shortened retroactive coverage, address verification, quarterly death master file matching
1 Jan 2028	CMS-2439-F appointment wait-time standards bite on California's calendar-year rating period
31 Dec 2028	Medicaid & CHIP Quality Rating System implemented
1 Jan 2029	Secret-shopper surveys validate network and wait-time data
1 Oct 2029	§71106 — eligibility error above 3% triggers disallowance , good-faith waiver restricted
30 Sep 2029	Superior Systems Waiver expires — the authority for FFS inpatient utilization management

Proposed, not final: CMS-0062-P, published 14 April 2026, would extend prior-authorization APIs to drugs with a proposed compliance date of 1 October 2027. **Any paper treating it as binding is wrong.** Note where it lands: on Medi-Cal Rx and Prime Therapeutics, on a different clock from the CA-MMIS obligations — and no single owner in the estate can absorb both.

CALIFORNIA

DATE	ITEM
1 Jan 2025 in force	SB 1120 — AI may not deny, delay or modify on medical necessity; determinations by a licensed professional. Binds plans and insurers, not DHCS fee-for-service
10 Feb 2026	BHIN 26-008 applies CMS-0057-F to mental health plans and drug Medi-Cal by state authority
12 May 2026	APL 26-008 — interoperability and prior-authorization requirements for managed care plans, superseding APL 22-013
1 Jul 2026 passed	Data Exchange Framework wave — emergency medical services and medical foundations
Oct 2026	Justice-involved reentry reaches its final statewide wave
31 Dec 2026	MCO tax expires. CalAIM 1115 demonstration and 1915(b) waiver expire. Renewals filed May 2026; CMS action pending as of 27 August 2026
1 Jul 2027	Data Exchange Framework closes on community and rural health clinics — the safety-net layer
1 Jan 2028	SB 306 — prior authorization retired for services approved at or above 90%. Applies to plans and insurers; excludes DHCS fee-for-service

THE CONVERGENCE

On **1 January 2027** DHCS must have running five new statutory determination processes and four interoperability APIs — in the same fiscal window in which its principal financing mechanism expires and its section 1115 authority lapses. There is no version of that date California staffs its way through. And the funder has written the preference into its own guidance: a state **must first attempt to use reliable information available to the state** before asking a person for a document.

Two dates that are not settled. CalAIM renewal and CMS approval of California's conforming managed-care tax were both pending at the time of writing. No paper may state either as approved.

§6 · THE MONEY

Match rates are not appropriations

This section exists because two of our papers priced the same federal match in opposite directions. This is the version that survives a Legislative Analyst.

WHAT THE FEDERAL MATCH ACTUALLY IS

RATE	FOR WHAT	CONDITIONS
90 / 10	Design, development, installation or enhancement of a mechanized claims processing and information retrieval system	42 CFR §433.112, subject to CMS approval of an advance planning document. Conditions include modularity, business rules separated from core programming in human- and machine-readable form, reuse, and no proprietary lock-in — plus §433.112(b)(11): the system must align to and advance in MITA maturity
75 / 25	Operation of an approved system	42 CFR §433.116. Post-certification only. Both rates confirmed unchanged in the current eCFR
50 / 50	Program administration	The default. This is where governance sits — promotion authority, sampling, shadow adjudication, drills, the oversight interface

THE SENTENCE EVERY PAPER MUST CONTAIN

90/10 and 75/25 are matching rates, not appropriations. California must appropriate the state share first, inside an advance planning document it must file anyway. The 75 applies only after certification. And the governance apparatus that makes any autonomy or evidence programme defensible is **program administration at 50/50** — which is usually the majority of the work being proposed.

The honest framing is therefore: *already at ninety cents on the federal dollar for the build, already inside a filing California must make, and at fifty cents for the governance.* It is not free, and a paper that implies otherwise loses the room in one sentence.

THE THREE-BUCKET TABLE EVERY PAPER SHOULD PUBLISH

Rather than a single number, split the ask: **(a)** design and development inside a named module at 90/10 · **(b)** maintenance and operations post-certification at 75/25 · **(c)** program administration at 50/50, with the non-federal source named. Then state the annual General Fund ask as a figure. Conceding the state share yourself is worth more than any argument about the ninety.

THE CERTIFICATION PATH

Streamlined Modular Certification replaced the older toolkits under SMD 22-001 and is built on measurable outcomes and metrics reported through operational reporting. The consolidated template set — including the Operational Report Workbook — became mandatory 1 July 2026. Any indicator a paper proposes should be expressed as an SMC outcome with a metric and a cadence, because that is the currency both SMC and CMS's successor maturity model accept.

THE FISCAL PINCER

The Legislative Analyst projects roughly **\$10bn in annual operating deficits through 2029-30**. The MCO tax expires 31 December 2026. The enacted 2026-27 Budget Act books a conforming replacement rising to **\$2.3bn**; the Analyst's own projection for the same instrument was in the **mid-tens of millions** — a spread of roughly \$2bn a year, resting on a federal approval that is unverified.

Present that as a **range, not a number**, and name both sources. And say plainly that the state share

§7 · INTEROPERABILITY

What is exchanged today, and what is owed by January 2027

The estate is not one system. It is at least six transaction domains with different intermediaries, different standards and different owners.

WHAT MOVES TODAY

FLOW	STANDARD AND TRANSPORT
FFS claim submission	ASC X12N 837 v5010 (P, I, D) through the Medi-Cal provider portal; paper CMS-1500 and UB-04 still accepted
Remittance	The remittance advice, produced by the State Controller's Office. Not an 835 to the provider in the commercial sense
Eligibility verification	Four channels, only one a standard transaction — X12 270/271, plus point-of-service and telephone channels
Managed care encounters	X12 837 and NCPDP to PACES over secure file transfer. The most precisely specified flow in the estate
Provider network data	X12 274 — the transaction most papers miss entirely
Capitation and enrollment	CAPMAN
Eligibility interfaces	CalSAWS ↔ CalHEERS over eHIT ; both ↔ MEDS; MEDS ↔ CA-MMIS
Federal reporting	T-MSIS to CMS

A FINDING, NOT A FOOTNOTE

There is **no single published index of California's companion guides**. They are distributed across the fiscal intermediary's site, the Medi-Cal Rx portal, PACES documentation and plan-facing letters. A state that cannot point to one place where its own interface specifications live has an interoperability problem before any new API is built.

TERMINOLOGY AND IDENTITY

There is no terminology authority in the estate, and that is the finding. No published owner for code systems and value sets, no terminology service. Provider identity spans NPI, PAVE and the provider master file with no single authoritative propagation event; member identity runs on the client index number and MEDS. This is precisely where "conformed data" is either real or rhetorical — and today it is rhetorical. A terminology service is a small, fundable, and currently absent module.

THE OBLIGATION NOBODY HAS STARTED

DHCS-as-fee-for-service-payer carries CMS-0057-F obligations **in its own right**. They attach to the single state agency for the FFS population and **cannot be delegated to plans**. Four are due 1 January 2027.

Against three of the four, and against every supporting artifact, **nothing has been published** — no design, no procurement, no attestation, no extension request. This is the most consequential fact in the document, and it is four months out.

DHCS-AS-FFS MUST HAVE	DUE	PUBLISHED
Patient Access API — prior-auth data added	1 Jan 2027	Base API live; <i>an email address is the entire developer interface</i>
Provider Access API + opt-out process	1 Jan 2027	Nothing
Payer-to-Payer API + opt-in process	1 Jan 2027	Nothing
Prior Authorization API	1 Jan 2027	Nothing
PA item-and-service list — the covered items requiring authorization, with required documentation	in-API by 1 Jan 2027	Not found anywhere
Denial reason on every PA response	1 Jan 2026	Operational
Public PA metrics, nine measures	31 Mar annually	Yes — three CY2025 reports
Extension or exemption request, if intended	before the date	No public indication one was sought

THREE ERRORS TO AVOID

Do not put a Provider Directory API in the DHCS column. It is CMS-9115-F and applies to managed care plans, not to Medicaid fee-for-service. This is a frequent error in payer whitepapers.

Do not treat the absence of a Medi-Cal Rx metrics report as a gap. Drugs are excluded from CMS-0057-F by definition; they are the subject of the proposed CMS-0062-P.

Do not assume TEFCFA is material here. It imposes no obligation on a state Medicaid agency acting as a payer, and CMS-0057-F does not reference it. Treat it as out of scope unless a specific use case brings it in.

§7.3

The interoperability horizon

What a 2032–34 target state would actually contain, expressed as components and the standard each exposes. **This is our construct, not a sourced roadmap** — but every component below is either mandated, procurable, or already half-built.

COMPONENT	EXPOSES
Prior authorization service	FHIR R4, US Core 6.1.0, SMART 2.0.0; Da Vinci CRD, DTR, PAS. Publishes the item-and-service list as data
Member and provider access	Patient Access and Provider Access APIs including bulk; a published capability statement and real developer documentation
Payer-to-payer exchange	Bulk FHIR, five-year lookback, opt-in managed as data
Terminology service	Versioned code systems and value sets with a named owner. Absent today
Entity resolution service	Member and provider identity with measured merge quality, published separately as false-merge and false-split rates
Evidence and provenance store	Per-decision record — inputs, rule version, decider, reason, appeal reference — state-owned and portable
Encounter reconciliation	Normalized rejection reason taxonomy, committed acknowledgment latency, published completeness metric

Each maps to a Streamlined Modular Certification outcome with a metric and a reporting cadence — which is how the horizon gets funded rather than merely drawn. **The state-owned pieces are the terminology service, entity resolution and the evidence store**; those are the ones that must survive a change of fiscal intermediary, and therefore the ones whose ownership must be written into a contract rather than assumed.

§8

The legal constraint register

Reproduce this box, identically, in every California paper. It is the floor.

Due process — the federal floor. 42 CFR §431.205(d) requires the hearing system to meet the standards of *Goldberg v. Kelly*. §431.210 requires "a clear statement of the specific reasons supporting the intended action." §431.211 requires notice at least ten days before the date of action. §431.240(a) requires an impartial decisionmaker not involved in the initial determination — which presupposes an identifiable initial determiner.

Managed care. §438.400(b) defines an adverse benefit determination. §438.406(b)(2) keeps the appeal separate from the decision it reviews. **§438.210(b)(3) requires that a denial, or an authorization of less than requested, be made by an individual with appropriate expertise.**

California — and get this right. SB 1120 does not bind DHCS fee-for-service. It amends Health & Safety Code §1367.01 and Insurance Code §10123.135 — health care service plans and disability insurers. DHCS as a state agency is neither, and the statute contains no provision reaching the Department's own TAR adjudication.

The constraint that does bind DHCS fee-for-service is California's own regulation. 22 CCR §51003 vests authorization in a designated Medi-Cal consultant and provides that the consultant's responsibilities **"shall not be delegated,"** except as provided under §§51013 and 51014. That non-delegation clause — not SB 1120 — is the real legal boundary on autonomous adjudication in the FFS estate. *The current text of §§51013 and 51014 should be retrieved before any paper relies on the exceptions.*

WHY THE CORRECTION MATTERS

A paper that anchors its California legal argument on SB 1120 while describing fee-for-service will be corrected by a DHCS lawyer in the first meeting. The §51003 argument is **stronger, more precise, and California's own** — and it produces the same design conclusion: the machine may assemble evidence and compute completeness, but the determination is a named consultant's act.

ALSO STANDING

42 CFR Part 2 — substance use disorder records, in force since 16 February 2026 — constrains how behavioral health data may move through any automated pipeline, and makes cross-party reconciliation structurally harder rather than merely more expensive

§9 · THE CAPABILITY MODEL

One model, and the traceability that makes it a backbone

Thirty-one business capabilities in six layers. This is the firm's California model; the taxonomies in the individual papers become views of it, not rivals to it.

Our construct, not a MITA artifact. The boundary rule: **EV holds facts and policy · OR moves work · DE exercises judgment · EN touches people · AS watches DE and OR · FI determines and moves money.** Two-letter prefixes are deliberate — single letters collide with the rung and tier vocabularies used in the individual papers.

EV · EVIDENCE & MEMORY

EV1 Entity Resolution
EV2 Longitudinal Record Assembly
EV3 Evidence Ledger
EV4 Policy-as-Code Registry
EV5 Decision Provenance & Replay
EV6 Consent & Minimum-Necessary
EV7 Disclosure Resistance

OR · ORCHESTRATION

OR1 Case & Work Orchestration
OR2 Handoff Contract Enforcement
OR3 Exception & Escalation
OR4 Autonomy Control Plane
OR5 Human Attention Allocation

DE · DECISIONING

DE1 Eligibility & Benefit Determination
DE2 Medical Necessity & Authorization
DE3 Claim & Encounter Adjudication
DE4 Anomaly & Integrity Scoring
DE5 Adverse-Effect Gate
DE6 Policy Change Simulation

EN · ENGAGEMENT

EN1 Member Interaction
EN2 Provider, Plan & County Interaction
EN3 Outreach & Deadline Rescue
EN4 Notice, Language & Accessibility

AS · ASSURANCE & GOVERNANCE

AS1 Autonomy Evidence Record
AS2 Continuous Control Monitoring
AS3 Appeals & Fair Hearing Support
AS4 Disparate-Impact Surveillance
AS5 Change Control & Rollback
AS6 Oversight & Audit Interface

FI · FINANCIAL

FI1 Rate & Capitation Administration
FI2 Payment Execution & Settlement
FI3 Recovery & Cost Avoidance
FI4 Federal Claiming & Match Defense

THE TRACEABILITY RULE — THIS IS WHAT MAKES IT A BACKBONE RATHER THAN A LIST

MITA AREA	PROCESS	CAPABILITY	SYSTEM TODAY	CONTRACT INSTRUMENT	ACCOUNTABLE OWNER	REACHABLE?
OM Operations	Process Claims	DE3 · EV3 · FI2	CA-MMIS	FI Business Ops + FI-M&O	DHCS CA-MMIS Operations	Yes — via contract
OM Operations	Process Encounters	EV2 · OR2 · FI1	PACES	State-run + plan contracts	DHCS Health Information Management	Yes — via plan contracts
CM Care Mgmt	Authorize Service	DE2 · DE5 · EV4	eTAR on the CA-MMIS portal	Rides the FI contracts	DHCS Clinical Assurance Division	Yes — DHCS adjudicates
PM Provider	Enroll / revalidate	EV1 · DE4	PAVE	SaaS; successor in procurement	DHCS Provider Enrollment Division	Yes — at re-procurement
EE Eligibility	Determine eligibility	DE1 · EV1 · EN3	CalSAWS · CalHEERS · MEDS	CalSAWS JPA · OTSI	58-county JPA	No — not DHCS's to start
CM Care Mgmt	Manage case (ECM)	EV2 · OR1 · EN1	Behavioral health estate	Consolidation unfunded and unscheduled	DHCS Behavioral Health / EDIM	Not yet — no platform

The right-hand column decides engagement scope. **Two of the six most-discussed Medi-Cal processes are not reachable by DHCS acting alone**, and no amount of architecture changes that.

§10 · THE RECORD

What went wrong elsewhere, and what it was actually caused by

Shared evidence. No paper needs to re-argue these; cite this section and move on.

CASE	WHAT HAPPENED	WHAT ACTUALLY CAUSED IT
Michigan MiDAS 2013–2015	Auto-adjudicated unemployment fraud from mismatches between claimant-reported earnings and employer wage records, flagging any discrepancy however trivial. Notice reached claimants only through a portal they had no reason to open, and non-response defaulted to an adverse determination . The Auditor General reviewed 22,000 determinations and found 93 percent involved no fraud.	Two parties asserting the same fact at different temporal granularities, with no reconciliation layer, no versioned precedence rule, and a contest channel that structurally did not reach the affected person.
Australia, Robodebt 2015–2019	Averaged an annual tax-office income figure across twenty-six fortnights, tested it against fortnightly reported income, raised a debt on the difference, and reversed the onus onto the recipient. Roughly 648,000 group members; about A\$1.763bn in debts withdrawn.	Averaging is a <i>conformance</i> operation. The enterprise successfully assembled a record and, in assembling it, fabricated a fact no source had asserted — then could not tell correct debts from incorrect ones, so all had to go.
Idaho <i>K.W. v. Armstrong</i> , 2016	A budget tool set annual Medicaid service funding for roughly 4,000 people with developmental disabilities, fitted on 733 of 3,512 participant records — the other 79 percent discarded as unreliable. Never recalibrated, never audited. Held unconstitutionally arbitrary.	No model improvement rescues a formula fitted on a corrupted remnant of its own population. The case to put in front of anyone proposing to solve an evidence problem with a better algorithm.
Arkansas & Indiana <i>Ledgerwood</i> 2017 · <i>Perdue</i> 2012	An algorithmic tool replaced nurse discretion in setting attendant-care hours; the courts held the state must explain the specific factors producing a cut. Indiana's automated eligibility processing issued denials carrying only generic reason codes.	Provenance and contestability, not data quality. Neither state could say <i>in a notice</i> which inputs produced the outcome. A rationale that cannot become a notice is not a rationale — and 42 CFR §431.210 binds California identically.
Tennessee TEDS <i>A.M.C. v. Smith</i> , 2024	The system failed for months to load Social Security benefits, disability status and marital status, so it could not evaluate disability-related coverage categories; during migration, enrollees were sorted into wrong households or dropped.	An assembled record, silently incomplete on exactly the fields that decide alternate-basis eligibility , with nothing detecting the incompleteness. It is ex parte redetermination run on evidence that could not bear it.
California's own CA-MMIS, 2010–2016	A fiscal intermediary contract valued at \$1.7bn, a takeover completed in 2011, one release delivered in December 2014, and a settlement in April 2016 abandoning development — the incumbent retained to operate and maintain what already existed through 2019.	Included because it is the reason a DHCS audience is sceptical of platform arguments, and because no California paper should propose a build without acknowledging it once, early, unprompted .

THE GENERALISATION THE RECORD SUPPORTS

Temporal misalignment between two sources describing one fact. An unusable population under a fitted rule. Provenance too thin to produce a lawful notice. Silently incomplete assembly presented as complete. **Across five jurisdictions and two decades, none of the documented catastrophes was a model failure.** In every one the automation was simple and deterministic, doing exactly what it was told, on evidence that could not bear the authority delegated to it.

AND THE GENERALISATION IT DOES NOT SUPPORT

This record is **not** an argument that public-benefits automation is inherently unsafe, and no paper should use it that way. It is an argument about *sequence*: that evidence quality is a precondition rather than a parallel workstream. A paper that cites these cases to counsel delay has misread them.

§11 · STANDING CAVEATS AND DEFERRED CANDIDATES

What no California paper may claim

DO NOT SAY

1. **"Funded by mandated spend the state already owes."**
There is no mandated spend and nothing is owed. Match rates are not appropriations — see §6.
2. **"Substantially autonomous operations in five years."** Not supported by any gate arithmetic we have published.
3. **Any current FFS claim volume, suspense rate, TAR volume by type, or California MITA self-assessment score.** None is public. Inventing one makes every other number suspect.
4. **Any dollar figure for a Medi-Cal modernization programme** not traceable to a budget document.
5. **That SB 1120 constrains DHCS fee-for-service.** It does not — §8.
6. **That the CalAIM renewal or the conforming MCO tax is approved.** Both were pending.
7. **That recoveries fund anything.** The federal share returns to CMS.
8. **That savings are General Fund savings.** DHCS does not hold the labour — §6.
9. **"MITA 3.0" as a name for a future state.** It is the current framework — §1.
10. **That CPPA automated-decision regulations bind DHCS.** They regulate businesses; DHCS is a state agency. The statute that does apply is the California Information Practices Act.

ALWAYS DISCLOSE

Where a reference implementation is cited, state plainly that it has never processed a Medi-Cal transaction, contains no California content, and is evidence of build method only. Put it where the reader cannot miss it, not in a methods note at the back.

DEFERRED CANDIDATES, AND WHY

Every paper should carry this paragraph so the client sees that the firm chose rather than defaulted.

CANDIDATE	WHY DEFERRED
Ex parte renewal	Wrong lane. Determination runs on CalHEERS and CalSAWS under county and joint-powers governance. DHCS's authority is indirect and no DHCS-side action reaches it
Reentry, Enhanced Care Management, behavioral health	The data estate is unconsolidated, unfunded and unscheduled, and 42 CFR Part 2 makes cross-party reconciliation structurally harder. Leading here means the instrument refutes the pilot
Clean-line claim adjudication as a standalone	The least interoperability-exposed process in the program, and the one with the weakest value case. Better used as the measurement instrumentation inside another use case
Provider enrollment and directory accuracy	Real, and the natural third given the 2029 secret-shopper marker — but it is a data-quality programme before it is anything else

THE FIRM'S CALIFORNIA ENTRY POINT

One first move, named once, so four papers stop proposing four.

WHY THAT ONE

It is on the anchor the direction named — Medi-Cal, fee-for-service, the CA-MMIS estate, DHCS-adjudicated end to end. Its business case is a compliance obligation rather than an invented saving. §51003 puts the legal boundary exactly where the design already sits: the machine assembles evidence and computes completeness; a named consultant determines. And it is the one place where the interoperability horizon and any autonomy argument are **the same build**.

On sources. Facts marked *confirmed* were retrieved from a primary source and linked in the working research. Facts marked *unverified* rest on secondary or aggregator sources and carry the mark wherever used. Items marked *DHCS to supply* are not published anywhere located and must not be estimated. The capability model in §9 and the horizon in §7.3 are this firm's construct and carry no external authority. Current as of 27 August 2026; the CalAIM renewal and the conforming managed-care tax were both pending on that date.